NCCP 00461 · VERSION 3

Your treatment, explained

Adults older than 65 with newly diagnosed glioblastoma who are not suitable for the standard radiotherapy regimen with temozolomide, under NCCP 00461. · NCCP 00461 v3 · Document revised 5 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Temozolomide with short-course radiotherapy, then adjuvant treatment

28-day cycle | Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Use your oncology unit's written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions. Contact the team promptly for a worsening headache, repeated vomiting, new cough or breathlessness, unusual bruising, yellow skin/eyes or symptoms that prevent drinking. Do not restart held chemotherapy without instructions.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: nccp · temodal · nccn-patient

Reference contents

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

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UNDERSTAND YOUR TREATMENT

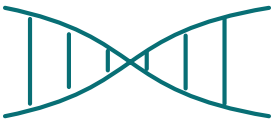
How your treatment works

This plan combines temozolomide with a shorter course of radiotherapy, followed by a break and adjuvant temozolomide, to help control newly diagnosed glioblastoma. NCCP identifies this particular indication as unlicensed; your team explains the evidence, alternatives and local consent process.

ORAL CHEMOTHERAPY CAPSULE

Temozolomide

Temodal / generic



Damages DNA in tumour cells

Temozolomide crosses into the brain and creates chemical changes in DNA. If the damage cannot be repaired, a cell may stop dividing or die. Healthy blood-forming cells can also be affected, which helps explain blood-count changes, infection and bleeding risks. Nausea prevention and liver monitoring are important too.

Capsule swallowed → DNA damaged → Cell growth disrupted

What to watch for

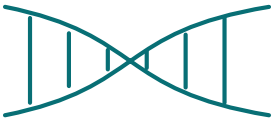
Report fever, bleeding, persistent vomiting, new breathing symptoms or yellow skin/eyes. Keep blood and liver tests even after capsule days finish.

Sources: nccp · temodal

TREATMENT FOCUSED ON THE BRAIN TUMOUR AREA

Radiotherapy

External-beam treatment planned by radiation oncology



Damages DNA in the treated area

Radiotherapy directs high-energy radiation at the tumour and a planned surrounding area. It aims to damage tumour-cell DNA while limiting dose to nearby healthy tissue. Fatigue, scalp irritation, hair loss and swelling can occur; longer-term effects depend on the treatment area and dose.

Individual planning scan → Radiation to planned area → Tumour-cell DNA damaged

What to watch for

Report new or worsening headache, vomiting, weakness, speech or memory change. The radiotherapy team gives a separate skin-care and symptom plan.

Sources: nccp · nccn-patient

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Brain / energy

Tumour / treatment

Fatigue or headache may occur. New seizures, sudden weakness, speech difficulty or loss of consciousness need emergency help.

3 Blood / infection

Temozolomide

Counts can fall after capsule days finish. Fever, sudden illness, bruising or bleeding needs urgent advice; keep all blood tests.

5 Liver

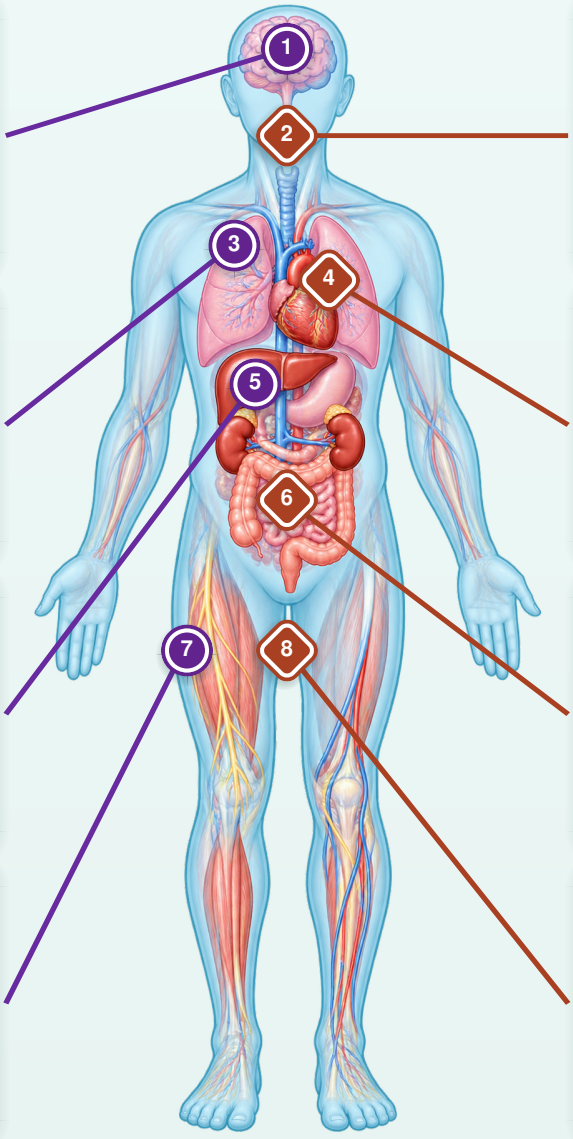
Temozolomide

Liver injury may appear during or after treatment. Attend liver tests and report yellow skin or eyes, dark urine or unusual itching.

7 Skin / scalp

Radiotherapy / temozolomide

Follow scalp care during RT; report new rash, blisters or peeling skin. Hair loss and skin irritation depend on the treatment received.



2 Mouth / eating

Temozolomide

Report mouth soreness, ulcers or reduced intake. Keep gentle mouth care and ask for help before eating or drinking becomes difficult.

4 Lungs / PJP

Temozolomide

New cough, fever or breathlessness needs prompt review. Follow the prescribed PJP-prevention plan during chemoradiotherapy.

6 Nausea / bowels

Temozolomide / support medicines

Take anti-sickness medicine as prescribed. Call for repeated vomiting, inability to drink, severe constipation or abdominal pain.

8 Fertility / pregnancy

Temozolomide

Discuss contraception and fertility before starting. Do not breastfeed during treatment. Ask for the written medicine-specific precautions.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

Radiotherapy phase → recovery → adjuvant cycles

DAILY CAPSULE PHASE

Temozolomide with three weeks of radiotherapy

Days 1–21 **Including weekends as prescribed**

Daily capsules and scheduled radiotherapy run together. Confirm the exact final capsule day and separate PJP-prevention plan.

RECOVERY AND REVIEW

Four-week break before adjuvant treatment

No routine temozolomide **Keep planned blood tests**

Radiotherapy has ended, but safety checks and prescribed support medicines continue. The team checks recovery before another prescription.

FIVE CAPSULE DAYS PER CYCLE

Up to six adjuvant 28-day cycles

D1–5 capsules **D6–28 no temozolomide**

The first cycle has its own prescription; the team may change the dose only after reviewing tolerance and blood counts. Six cycles are the current NCCP plan.

The team confirms each phase and prescription. Do not use the capsule dose or test schedule from a different phase.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Safety tests, symptoms and response review

The team books and reviews each test. Mandatory medicine-safety checks are distinct from individually timed MRI and tumour-tissue tests. There is no universal blood tumour marker for these treatment cycles.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00461 v3 · current Irish regimen · NCCN Guidelines for Patients: Brain Cancer Glioma, 2026 · patient education reference · EMA Temodal product information and patient leaflet

THE WHOLE TREATMENT JOURNEY

Temozolomide with short-course radiotherapy, then adjuvant treatment: treatment and checks

Adults older than 65 with newly diagnosed glioblastoma who are not suitable for the standard radiotherapy regimen with temozolomide, under NCCP 00461.

Take temozolomide daily for 21 days during short-course radiotherapy, including weekends as prescribed. After a four-week break, take temozolomide on Days 1–5 of each 28-day cycle for up to six adjuvant cycles.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits / recovery	Next-cycle / response review	After a pause or stop
Blood count / kidneys / liver NCCP 00461	FBC, renal/liver profile, glucose, HBsAg and anti-HBc	FBC weekly during three-week RT	Continue FBC weekly for four weeks after RT; renal/LFT at 1 and 4 weeks after RT	Day 1 FBC/renal/LFT; cycle 1 can use four-week post-RT review. EMA Day 22 FBC	LFT after cycles; follow delayed toxicity after the last dose
Neurology / medicines / function NCCP 00461	Record neurological symptoms, daily function and current medicines	Review seizures, headache, cognition, mood, walking and any steroid plan	Report changes early; do not wait for the next MRI	Review treatment tolerance, medication support and patient goals	Agree follow-up and rehabilitation or symptom support
MRI / diagnosis NCCP 00461	Confirm integrated diagnosis, molecular results and baseline imaging	MRI timing according to the neuro-oncology plan	Keep the booking and result-review contact in your passport	Interpret scan with symptoms, treatment effects and clinical review	Agree the next imaging and clinical review

What each check is for

1

Temozolomide with three weeks of radiotherapy

Daily capsule phase

This plan combines temozolomide with a shorter course of radiotherapy, followed by a break and adjuvant temozolomide, to help control newly diagnosed glioblastoma. NCCP identifies this particular indication as unlicensed; your team explains the evidence, alternatives and local consent process.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00461

2

Four-week break before adjuvant treatment

Recovery and review

This plan combines temozolomide with a shorter course of radiotherapy, followed by a break and adjuvant temozolomide, to help control newly diagnosed glioblastoma. NCCP identifies this particular indication as unlicensed; your team explains the evidence, alternatives and local consent process.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00461

3

Up to six adjuvant 28-day cycles

Five capsule days per cycle

This plan combines temozolomide with a shorter course of radiotherapy, followed by a break and adjuvant temozolomide, to help control newly diagnosed glioblastoma. NCCP identifies this particular indication as unlicensed; your team explains the evidence, alternatives and local consent process.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00461

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.38

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Take temozolomide daily for 21 days during short-course radiotherapy, including weekends as prescribed. After a four-week break, take temozolomide on Days 1–5 of each 28-day cycle for up to six adjuvant cycles.

Treatment timing: displayed days 1–28

Treatment or check	Days in this window	What happens
Temozolomide once daily	Days 1–5	Use the current phase prescription, fasting and with the prescribed nausea plan.
No temozolomide capsules	Days 6–28	Recovery, symptoms and blood/liver tests; restart only when authorised.
Day 22 blood count	Day 22	EMA recommends a count around Day 22 and follow-up until recovery.
Safety throughout the cycle	Days 1–28	Fever, bleeding, neurological or breathing symptoms need prompt advice.

Dose changes belong to the current phase: Daily chemoradiotherapy and five-day cycles use different doses. Check the label and phase with the pharmacist whenever a new supply is issued.

Effects can be delayed: Blood counts and liver effects can change after capsule days finish or after the last cycle. Keep the follow-up tests and emergency plan.

No replacement after vomiting: If you vomit after a temozolomide dose, do not take a second dose that day. For a missed dose, follow the pharmacist's written instructions or contact the team; never double up.

> **Why these bands stay broad:** This 28-day view is the ADJUVANT phase only. The earlier daily chemoradiotherapy phase and four-week break are shown separately above. Symptoms can occur on any day. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00461 v3 · current Irish regimen ↗

> **There are no guaranteed "safe days".** Use your oncology unit's written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions.

Complete timing and symptom notes

Day 1–5: Five daily capsule doses

Check the prescribed capsule strengths and number each day. Take fasting with the anti-sickness plan. Do not use leftover capsules to change the dose or replace a vomited dose.

Day 6–21: No temozolomide; recovery continues

Rest days allow recovery but blood counts can still fall. Continue other medicines only as individually prescribed. Contact the team for fever, sudden illness, bruising or persistent vomiting.

Day 22–28: Blood count and next-cycle decision

Have the Day 22 blood count and any follow-up blood tests arranged. Before the next cycle, the team reviews counts, kidney/liver tests and symptoms. Do not start the next cycle just because the calendar reaches Day 1.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Taking the treatment

Take the exact temozolomide capsule strengths and number on your pharmacy label, once daily when prescribed, on an empty stomach. Swallow whole with water; do not open, chew or crush. Ask the pharmacist to write the fasting interval and anti-sickness timing. Different phase doses are not interchangeable.



STEP 2

Missed or vomited medicine

If you vomit after temozolomide, do not take another dose that day. For a missed dose, use your pharmacist’s written instructions or call before taking it late; do not double a dose. Never replace a dose held for toxicity.

STEP 3

Medicine and supplement review

Bring an up-to-date list of all medicines, including steroids, seizure medicines, anticoagulants, non-prescription treatments, vitamins and herbs. Ask before starting something new. Keep a record of all dose changes.

STEP 4

Storage and handling

Keep capsules in their original labelled packs, away from children. Do not open, crush or chew. If swallowing or medicine organisation is difficult, contact the team before the next dose and agree safe help. Return unused chemotherapy to the pharmacy.

STEP 5

Pregnancy, breastfeeding and fertility

Treatment can harm a pregnancy and may affect fertility. Discuss fertility preservation, contraception for patients and partners, and the required period after the final dose before starting. Do not breastfeed during treatment. Ask the team for written, medicine-specific advice.

STEP 6

Appointments and transport

Before starting: full blood count, kidney and liver tests, glucose and hepatitis B screening. Blood counts weekly through weeks 1–7, including four weeks after the last radiotherapy. Kidney and liver tests one and four weeks after radiotherapy. Adjuvant Day 1 blood count, kidney and liver tests each 28-day cycle; the four-week post-RT tests can form cycle 1 assessment. EMA additionally advises Day 22 blood count and after-cycle liver tests. MRI timing is individual.

STEP 7

Help with the daily plan

Ask a trusted person to help check the calendar if memory, vision, fatigue or hand strength makes treatment difficult. Confirm who is responsible for checking doses, collecting medicine, transport and calling for help. Keep consent and personal preferences central.

Sources: NCCP 00461 v3 · current Irish regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Nausea prevention

Temozolomide has moderate-to-high oral emetic potential. Take the prescribed anti-sickness plan before chemotherapy and use rescue medicine as instructed. Ask what to do if nausea prevents taking medicines or fluids.

2 PJP infection prevention

PJP-prevention medicine is required during temozolomide with radiotherapy in this NCCP regimen. Your team supplies the prescription and decides how long to continue, including if lymphocyte counts remain low. Do not stop it just because radiotherapy has finished.

3 Bowels, food and fluids

Anti-sickness medicines and other medicines can worsen constipation. Ask for your bowel plan, and call for persistent vomiting, severe constipation or difficulty drinking. Do not double a dose after vomiting.

4 Steroid and seizure plans

Keep steroids and seizure medicines on their own written schedules if prescribed. They are not routine medicines for every patient, and chemotherapy rest days do not automatically mean stopping them. Ask the pharmacist to check interactions.

Sources: nccp · temodal

TREATMENT REFERENCE · 01

Your treatment and its purpose

Use the regimen and phase agreed with your team.

Why this treatment?

This plan combines temozolomide with a shorter course of radiotherapy, followed by a break and adjuvant temozolomide, to help control newly diagnosed glioblastoma. NCCP identifies this particular indication as unlicensed; your team explains the evidence, alternatives and local consent process.

Sources: nccp · temodal

What each part does

Temozolomide crosses into the brain and creates chemical changes in DNA. If the damage cannot be repaired, a cell may stop dividing or die. Healthy blood-forming cells can also be affected, which helps explain blood-count changes, infection and bleeding risks. Nausea prevention and liver monitoring are important too. Radiotherapy directs high-energy radiation at the tumour and a planned surrounding area. It aims to damage tumour-cell DNA while limiting dose to nearby healthy tissue. Fatigue, scalp irritation, hair loss and swelling can occur; longer-term effects depend on the treatment area and dose.

Sources: nccp · temodal · nccn-patient

Schedule and planned duration

Take temozolomide daily for 21 days during short-course radiotherapy, including weekends as prescribed. After a four-week break, take temozolomide on Days 1–5 of each 28-day cycle for up to six adjuvant cycles.

Sources: nccp

Evidence and uncertainty

The addition of temozolomide improved survival in the study population. The study's age criterion and treatment duration do not replace the current Irish indication and six-cycle adjuvant plan. An MGMT result helps discuss benefit but does not independently authorise or exclude treatment in this patient guide. Trial results do not predict individual survival.

Sources: perry

TREATMENT REFERENCE · 02

Taking treatment safely

The exact prescription and calendar matter.

Taking the treatment

Take the exact temozolomide capsule strengths and number on your pharmacy label, once daily when prescribed, on an empty stomach. Swallow whole with water; do not open, chew or crush. Ask the pharmacist to write the fasting interval and anti-sickness timing. Different phase doses are not interchangeable.

Sources: nccp · temodal

Missed or vomited medicine

If you vomit after temozolomide, do not take another dose that day. For a missed dose, use your pharmacist's written instructions or call before taking it late; do not double a dose. Never replace a dose held for toxicity.

Sources: nccp · temodal

Medicine and supplement review

Bring an up-to-date list of all medicines, including steroids, seizure medicines, anticoagulants, non-prescription treatments, vitamins and herbs. Ask before starting something new. Keep a record of all dose changes.

Sources: nccp · temodal

Storage and handling

Keep capsules in their original labelled packs, away from children. Do not open, crush or chew. If swallowing or medicine organisation is difficult, contact the team before the next dose and agree safe help. Return unused chemotherapy to the pharmacy.

Sources: nccp · temodal

Pregnancy, breastfeeding and fertility

Treatment can harm a pregnancy and may affect fertility. Discuss fertility preservation, contraception for patients and partners, and the required period after the final dose before starting. Do not breastfeed during treatment. Ask the team for written, medicine-specific advice.

Sources: nccp · temodal

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Before starting: full blood count, kidney and liver tests, glucose and hepatitis B screening. Blood counts weekly through weeks 1–7, including four weeks after the last radiotherapy. Kidney and liver tests one and four weeks after radiotherapy. Adjuvant Day 1 blood count, kidney and liver tests each 28-day cycle; the four-week post-RT tests can form cycle 1 assessment. EMA additionally advises Day 22 blood count and after-cycle liver tests. MRI timing is individual.

Sources: nccp · temodal

Help with the daily plan

Ask a trusted person to help check the calendar if memory, vision, fatigue or hand strength makes treatment difficult. Confirm who is responsible for checking doses, collecting medicine, transport and calling for help. Keep consent and personal preferences central.

Sources: nccn-patient

TREATMENT REFERENCE · 03

Tests, urgent symptoms and follow-up

Keep planned tests and know how to get help.

Your monitoring plan

Before starting: full blood count, kidney and liver tests, glucose and hepatitis B screening. Blood counts weekly through weeks 1–7, including four weeks after the last radiotherapy. Kidney and liver tests one and four weeks after radiotherapy. Adjuvant Day 1 blood count, kidney and liver tests each 28-day cycle; the four-week post-RT tests can form cycle 1 assessment. EMA additionally advises Day 22 blood count and after-cycle liver tests. MRI timing is individual.

Sources: nccp · temodal

When to seek help

Use your oncology unit's written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions. Contact the team promptly for a worsening headache, repeated vomiting, new cough or breathlessness, unusual bruising, yellow skin/eyes or symptoms that prevent drinking. Do not restart held chemotherapy without instructions.

Sources: nccp · temodal · nccn-patient

Support medicines

Temozolomide has moderate-to-high oral emetic potential. Take the prescribed anti-sickness plan before chemotherapy and use rescue medicine as instructed. Ask what to do if nausea prevents taking medicines or fluids. PJP-prevention medicine is required during temozolomide with radiotherapy in this NCCP regimen. Your team supplies the prescription and decides how long to continue, including if lymphocyte counts remain low. Do not stop it just because radiotherapy has finished. Anti-sickness medicines and other medicines can worsen constipation. Ask for your bowel plan, and call for persistent vomiting, severe constipation or difficulty drinking. Do not double a dose after vomiting. Keep steroids and seizure medicines on their own written schedules if prescribed. They are not routine medicines for every patient, and chemotherapy rest days do not automatically mean stopping them. Ask the pharmacist to check interactions.

Sources: nccp · temodal · nccn-patient

A delay is a treatment decision

Blood counts, organ tests or symptoms may mean a dose is held, reduced or stopped. This is a safety decision, not a reason to take extra capsules later. Ask which medicines change, which support medicines continue and when the next assessment is due.

Sources: nccp · temodal

After the final dose

Agree follow-up blood tests, symptom review, MRI timing and who to contact. Continue to report new problems; effects can outlast chemotherapy. Keep your medicine list and written plan current.

Sources: nccp · temodal

TREATMENT REFERENCE · 04

Understanding your particular glioma

Your diagnosis includes the tumour's cells, molecular findings and behaviour, not just the word brain cancer.

The name and location

Gliomas begin in supporting cells of the brain or spinal cord. Astrocytoma, oligodendroglioma and glioblastoma are different diagnoses. A tumour that spread to the brain from another organ is a brain metastasis and is not covered by these glioma regimens. Record the exact integrated diagnosis and where it is located.

Sources: nccn-patient

Grade is not the same as stage

Grade describes features linked to how a tumour behaves. Adult gliomas are usually discussed by their type, grade and molecular findings rather than the stage system used for many other cancers. Grade 4 does not mean cancer has spread through the body. Ask your specialist to explain your own report and any older terminology in the treatment protocol.

Sources: nccn-patient · nci-cns

IDH and 1p/19q

IDH1 or IDH2 changes and loss of parts of chromosomes 1p and 19q help classify particular gliomas. The combination of findings matters: one result should not be read in isolation. These tests are usually performed on tumour tissue. They do not automatically mean a change has been inherited or that relatives need testing.

Sources: nccn-patient

MGMT promoter methylation

MGMT is involved in repairing certain DNA damage. Methylation of its promoter can reduce that repair activity and helps the team discuss the likely benefit of temozolomide in relevant settings. It is not a blood tumour marker, a percentage of tumour left, or an individual survival prediction. Record the reported result and ask how it affects your treatment choice.

Sources: nccn-patient

A useful diagnosis record

Keep the biopsy or operation date, pathology wording, grade, IDH result, 1p/19q result and MGMT result if tested. Mark a result as pending or not applicable rather than assuming it is negative. Ask whether specialist pathology review or further testing would change the plan.

Sources: nccn-patient

TREATMENT REFERENCE · 05

Tests and scans: what each one tells you

Safety blood tests, tumour tissue tests and MRI answer different questions.

Safety blood tests

A full blood count checks infection-fighting white cells, oxygen-carrying red cells and platelets that help stop bleeding. Kidney and liver tests help assess treatment safety. Attend the regimen's required tests even when you feel well; a reassuring scan does not replace them. The schedule in this guide separates checks before, during and after treatment.

Sources: nccp · nccn-patient

MRI and neurological review

MRI is the main scan used to assess many gliomas. Your team also considers symptoms, strength, movement, speech, memory, daily function and medicines such as steroids. Tell staff before MRI about implants or metal, kidney problems, previous contrast reactions or difficulty lying still. Ask about support if scans make you anxious.

Sources: nccn-patient

Changes on a scan need interpretation

After treatment, inflammation and other treatment effects can resemble tumour growth on MRI. This is sometimes called pseudoprogression. The team may need comparison scans, specialist imaging or further assessment before deciding what a change means. New or worsening symptoms still need prompt review; do not assume they are harmless treatment effects.

Sources: nccn-patient

An individual follow-up timetable

Write the next MRI date, what question it should answer and who will explain the result. Timing depends on your diagnosis, treatment phase and previous findings. General follow-up intervals in an international booklet are not automatically your schedule. These regimens do not require a generic CT chest/abdomen/pelvis or a blood tumour marker before every cycle.

Sources: nccn-patient

Keep results with decisions

Use the passport to distinguish a test being booked, completed and reviewed. Record the agreed next step and whom to call if an appointment or result is missing. Do not alter chemotherapy or steroids yourself in response to a number or scan report.

Sources: nccn-patient

TREATMENT REFERENCE · 06

Brain symptoms, medicines and when to call

Symptoms may reflect the tumour, swelling, treatment, another medicine or a separate illness.

Know your usual symptoms

Write down your usual headache, strength, walking, speech, vision, memory, mood, sleep and seizure pattern before treatment. Note new changes, when they started and how they affect daily tasks. With your permission, a trusted person can help spot changes or record what happened.

Sources: nccn-patient

Urgent neurological changes

Use your oncology unit’s written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions.

Sources: nccp · nccn-patient

Steroids need a separate written plan

Steroids may be prescribed to reduce swelling around the tumour. They are not automatically needed for everyone. Ask for the exact dose, time, review date and any taper plan. Do not stop or change them suddenly without advice. Report sleep or mood changes, increased thirst, infection symptoms or muscle weakness and ask which additional checks or protection you need.

Sources: nccn-patient · nhs-dexamethasone

Seizure medicines and rescue instructions

If prescribed, take seizure medicines on the agreed schedule. Ask what to do after a missed or vomited dose and whom to contact for breakthrough symptoms. Some medicines can affect mood, alertness or interact with treatment. Do not start, stop or switch them yourself. A rescue medicine needs an individual plan and training for whoever may give it.

Sources: nccn-patient

Safety at home and outside

Ask your clinical team about driving, work, bathing, swimming, heights and machinery in light of your symptoms and any seizures. Local medical and licensing rules need individual advice; this guide does not give a universal driving clearance date. Occupational therapy can help adapt daily routines and the home.

Sources: nccn-patient

TREATMENT REFERENCE · 07

Your overall treatment plan and choices

Treatment should account for the tumour and the life you want to lead.

Understand the sequence

Surgery or biopsy, radiotherapy, chemotherapy and follow-up have different jobs. Ask your team to draw the intended sequence and identify where you are now. Concurrent treatment means treatments overlap; adjuvant treatment follows the main treatment. A change of phase may change the dose, blood tests and support medicines.

Sources: nccn-patient

Agree what treatment is trying to achieve

Ask whether the aim is to control the tumour, delay growth, relieve symptoms, preserve function or a combination of these. Discuss likely benefits, burdens, alternatives and what would prompt a pause or stop. The answer depends on the exact diagnosis, previous treatment and your priorities.

Sources: nccn-patient

Put evidence in context

Clinical trials describe groups of people and may use older pathology names or different schedules. They cannot predict one person’s response or lifespan. Ask how closely the evidence matches your tumour and treatment. No benefit number from one schedule should automatically be applied to another.

Sources: nccn-patient

Discuss options available to you

Ask about appropriate clinical trials, specialist opinions, symptom support and other treatment options. A medicine or device described in an international guide is not automatically licensed, funded or suitable in Ireland. The treating multidisciplinary team explains what is available and relevant.

Sources: nccn-patient

Make room for your priorities

Bring up fatigue, thinking, independence, family time, work, travel and side effects you most want to avoid. Ask for information at a pace and in a format that suits you. You may ask for a trusted person to join the discussion, written notes and time to reconsider decisions.

Sources: nccn-patient

TREATMENT REFERENCE · 08

Living well, rehabilitation and support

Practical and symptom support can begin alongside anticancer treatment.

Thinking, communication and daily tasks

Tell the team about word-finding, attention, memory, reading, vision or planning difficulties. Rehabilitation may include physiotherapy, occupational therapy, speech and language therapy, or neuropsychological support. Written routines, rest breaks and help with medicine organisation can make day-to-day care more manageable.

Sources: nccn-patient

Food, nausea, bowels and energy

Report poor appetite, weight change, vomiting or constipation early. Ask for dietitian help if eating is difficult. Use the prescribed anti-sickness and bowel plan; do not wait until symptoms are severe. Choose activity that is safe for your balance and strength and balance it with rest. Ask about treatable causes of fatigue.

Sources: nccp · nccn-patient

Emotional support and carers

Anxiety, low mood, changes in behaviour and uncertainty can affect both patients and carers. Tell the team what has changed and ask for appropriate psychological or social support. Carers may need their own information, practical help and breaks. Agree who the team may share information with.

Sources: nccn-patient

Work, transport and finances

Ask the specialist nurse or social worker about getting to treatment, time off work, benefits, caring responsibilities and support at home. Plan who can help with daily capsules and appointments if your memory, vision or fatigue makes these difficult.

Sources: nccn-patient

Supportive care and future wishes

Supportive and palliative care can help with symptoms and quality of life while active treatment continues. You can discuss future care preferences and who you want involved in decisions at a time that suits you. Ask the team for Irish advice and documentation rather than using legal or hospice rules from a US booklet.

Sources: nccn-patient

Information that works on paper

Keep the printed schedule, current medicine list, tests, contact numbers and emergency instructions together. This handbook can be read in sequence without internet access. The wallet card and optional QR code help others find the regimen information; they do not replace your prescription or emergency plan.

Sources: nccn-patient

Perry trial: short-course radiotherapy with temozolomide in older adults

A randomised study compared short-course radiotherapy alone with the addition of concurrent and adjuvant temozolomide in older adults with glioblastoma.

The addition of temozolomide improved survival in the study population.

Sources: perry

PREPARE FOR THE CONVERSATION

Questions worth asking

What is my exact diagnosis, grade and relevant molecular result?

Ask for the integrated pathology wording and an explanation of IDH, 1p/19q and MGMT where relevant. The diagnosis and treatment history guide how this regimen applies to you.

Which phase and capsule calendar am I on now?

Take temozolomide daily for 21 days during short-course radiotherapy, including weekends as prescribed. After a four-week break, take temozolomide on Days 1–5 of each 28-day cycle for up to six adjuvant cycles.

What is my PJP-prevention plan, including after radiotherapy?

The NCCP concurrent treatment requires PJP prevention. Ask which medicine, when to take it, monitoring and when the team will decide it can stop. Low lymphocyte counts may mean continuing longer.

Which tests must happen before I can take the next dose?

Before starting: full blood count, kidney and liver tests, glucose and hepatitis B screening. Blood counts weekly through weeks 1–7, including four weeks after the last radiotherapy. Kidney and liver tests one and four weeks after radiotherapy. Adjuvant Day 1 blood count, kidney and liver tests each 28-day cycle; the four-week post-RT tests can form cycle 1 assessment. EMA additionally advises Day 22 blood count and after-cycle liver tests. MRI timing is individual.

Who helps with seizures, steroids, memory or daily tasks?

Ask for a named contact, written seizure and steroid plans if prescribed, and rehabilitation or practical support. Agree who can help you organise treatment.

Which symptoms mean contact the team today?

Use your oncology unit's written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions. Contact the team promptly for a worsening headache, repeated vomiting, new cough or breathlessness, unusual bruising, yellow skin/eyes or symptoms that prevent drinking. Do not restart held chemotherapy without instructions.

When will we review the benefit and my priorities?

Ask for the next clinical and MRI review, how treatment effects will be distinguished from growth, and what would lead to continuing, changing or stopping treatment.

> **Try explaining the plan back:** Can you show the days for each medicine, the current phase, your next blood tests and whom to call for fever, vomiting or new neurological symptoms?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00461 v3. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00461 v3 · current Irish regimen
- NCCN Guidelines for Patients: Brain Cancer Glioma, 2026 · patient education reference
- EMA Temodal product information and patient leaflet
- Perry trial: short-course radiotherapy with temozolomide in older adults
- NCI: adult central nervous system tumours · diagnosis and grade education
- NHS: dexamethasone · use and stopping safely

Written source addresses

https://healthservice.hse.ie/documents/6977/461_V3_Temozolomide_RT_and_Adjuvant_Therapy_greater_than65_years_.pdf

https://www.nccn.org/patients/guidelines/content/PDF/brain-gliomas-patient.pdf

https://www.ema.europa.eu/en/documents/product-information/temodal-epar-product-information_en.pdf

https://pubmed.ncbi.nlm.nih.gov/28296618/

https://www.cancer.gov/types/brain/patient/adult-brain-treatment-pdq

https://www.nhs.uk/medicines/dexamethasone-tablets-and-liquid/how-and-when-to-take-dexamethasone-tablets-and-liquid/